



Gavin Newsom, Governor
State of California
Health and Human Services Agency
DEPARTMENT OF MANAGED HEALTH CARE
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March 3, 2026

VIA ELECTRONIC MAIL

Karen Choate
Blue Cross of California
1121 L Street, Suite 500
Sacramento, CA 95814

Re: Proposed Contract Termination with Henry Mayo Newhall Hospital
Block Transfer Filing No. 2026-HOSP-256
Filed on March 2, 2026

Dear Ms. Karen Choate,

The Department of Managed Health Care (Department) is reviewing the information in the above-referenced Block Transfer Filing (Filing) submitted by **Blue Cross of California** (Plan) for compliance with the requirements of the Knox-Keene Health Care Service Plan Act of 1975, as amended (Cal. Health & Safety Code § 1340 *et seq.*) and the regulations promulgated thereunder (Cal. Code Regs., Tit. 28, § 1000 *et seq.*).

In the event of contract termination, the Filing proposes to remove **Henry Mayo Newhall Hospital** (Terminating Hospital) from the Plan's hospital network. The effective date of contract termination between the Terminating Hospital and the Plan (Termination Date) is currently **April 1, 2026**.

Please review the following comments and feel free to contact me if you would like to discuss any of the issues before filing the Plan's response.

1. Please confirm that the Plan is aware that it cannot transition its enrollees or send Enrollee Transfer Notices without the Department's approval.

Plan Response1: The Plan will not transition its enrollees or send Enrollee Transfer Notices without the Department's approval.

2. Please confirm whether the hospital termination is part of a health system termination. If yes, please confirm the health system and whether the contract is a stand-alone contract.

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Plan Response2: The hospital termination is Not part of a health system termination.

3. Please confirm whether the contract termination between the Plan and Henry Mayo Newhall Hospital impacts Point of Service (POS) enrollees.

Plan Response3: The contract termination between the Plan and the Terminating Provider impacts Point of Service (POS) enrollees.

4. For each Alternate Hospital, please summarize or confirm the following:
 - a. The Plan contacted the hospital regarding the Block Transfer,
 - b. Any concerns expressed by the hospital regarding its ability to accommodate the hospital-based needs of enrollees redirected by the Block Transfer,
 - c. Whether the Plan has sent or received a notice of contract termination from the Alternate Hospitals, and if so, whether that contract is set to terminate within the next 6 months,
 - d. Contract termination date,
 - e. Whether the contract is evergreen and confirm whether each evergreen contract will automatically renew.

Plan Response4: The Plan outreaches to the hospitals with occupancy rates at 80% or higher and documented any concerns and capacity response(s) in the filing portal. For hospitals with occupancy rates <80%, justification is not provided as to the sufficiency of the Alternate Hospital's capacity to serve additional plan enrollees per §1300.67.1.3(2)(L): *"Based upon the data made public on the Office of Statewide Health Planning and Development's website, for each of the proposed Alternate Hospitals, provide the available bed occupancy rate for the most recently completed calendar year. If the rate is at 80% or higher, please provide justification as to the sufficiency of the Alternate Hospital's capacity to serve additional plan enrollees."* The Plan has not sent or received notice of contract terminations from the Alternate Hospitals. The contracts are evergreen and will automatically renew.

5. In the event of contract termination where the Plan removes the Terminating Hospital from its contracted provider network, please summarize or confirm the following that may result:
 - a. The Plan's process for ensuring compliance with Cal. Code Regs., Tit. 28, § 1300.51(d)(H)(ii). (ii) Hospitals. In the case of a full-service plan, all enrollees have a residence or workplace within 30 minutes or 15 miles of a contracting or plan-operated hospital which has a capacity to serve the

entire dependent enrollee population based on normal utilization, and, if separate from such hospital, a contracting or plan-operated provider of all emergency health care services.

Plan Response5: Members who reside beyond 30 minutes or 15 miles of a contracted or plan-operated hospital will be allowed to access the terminating/terminated hospital as appropriate, reimbursed at in-network benefit levels. Members may access emergency services at any hospital, including the terminating/terminated hospital; coverage is provided according to each member's policy benefits.

6. The Plan identifies a total of 2,758 enrollees who will travel beyond 15 miles or 30 minutes from his/her PCP post-termination. For the ZIP codes in the table below, please confirm whether the Plan has alternative access standards approved by the Department (including the filing number and approval date).

ZIP Code	Number of Enrollees
91354	1,375
93532	1,383

PlanResponse6: The number of enrollees identified a total of 1,383 enrollees who will travel beyond 15 miles or 30 minutes from his/her PCP post-termination, as follows (the Plan inadvertently noted the total of 1,383 for 93532 instead of 8):
:

ZIP Code	Number of Enrollees
91354	1,375
93532	8
Total	1,383

For only ZIP code 93532, the Plan has alternative access standards approved by the Department per filing number 20243408 approved on 10/11/2024. The Plan will submit request for alternate access standards for zip 91354 post termination per the Department's instructions.

7. The Plan's PPO line of business is licensed by the Department. Please confirm the following:
- The Plan will send an Enrollee Transfer Notice approved by the Department to all PPO enrollees with a 15 mile radius of the Terminating Hospital.

Plan Response7: The Plan will send Department-approved Enrollee Transfer Notices to all PPO enrollees residing within a 15-mile radius of the Terminating Hospital.

8. For each affected provider group that primarily utilizes the Terminating Hospital, please summarize or confirm the following:
 - a. The ability of the provider group to redirect enrollees to the Alternate Hospitals:
 - i. Whether there is any specialty for which all of the associated specialists have admitting privileges **only** at the Terminating Hospital and not at any of the Alternate Hospitals,
 1. If yes, please identify the specialty and explain how Plan enrollees assigned to the provider group will receive care and services for that specialty following contract termination.
 - ii. If the provider group does not have any primary care physicians or specialists with admitting privileges to one or more Alternate Hospitals, please explain how its assigned enrollees will be admitted to the Alternate Hospital(s) (e.g. via hospitalists, Plan admission process, etc.).
 1. If hospitalists are utilized, please indicate:
 - a. The name of the group,
 - b. The number of associated hospitalists, and
 - c. If the hospitalists can admit enrollees for any medically necessary services.

Plan Response8: Provider groups have the full scope of specialty services required by the Plan's contract with its enrollees. For credentialing and contracting purposes, each participating provider group contracted with the Plan must have a full array of specialists, including primary and high-volume specialties, available or accessible to serve our members. The provider groups meet this requirement by either employing and/or contracting with sub-specialists when necessary. No primary or sub-specialties were identified as lacking. If, for some reason, when an In-Network physician or other health care professional specialty is not available or accessible to an enrollee, the enrollee may request an Out-of-Network referral. Furthermore, providers shall arrange for the provision of specialty services from specialists outside the provider's contracted network if unavailable within the network, when medically necessary for the enrollee's condition. Enrollee costs for medically necessary referrals for Out-of-Network providers will not exceed In-Network co-payments, co-insurance, and deductibles.

9. Please confirm whether the HMO providers that routinely utilize the Terminating Hospital have adequate admitting privileges for PCPs and specialists to at least

one of the identified Alternate Hospitals containing all services that will be lost should the contract ultimately terminate.

Plan Response9: HMO providers that routinely utilize the Terminating Hospital have adequate admitting privileges for PCPs and specialists to at least one of the identified Alternate Hospital containing all services.

10. Please confirm whether the provider group(s) in this Block Transfer Filing are facing potential contract termination(s) within six months. In addition, if the Plan is required to file a Block Transfer Filing for any of the provider group(s), please provide the name of the provider group and the filing number of each Block Transfer Filing.

Plan Response10: The provider group(s) in this Block Transfer Filing are not facing potential contract termination(s) within six months.

11. Please confirm the medical groups that do not utilize the Terminating Hospital as a primary hospital, but have admitted enrollees for the most recently completed calendar year, are able to refer their enrollees to other hospitals within the Plan's network for services comparable to those provided by the Terminating Hospital.

Plan Response11: The medical groups that do not utilize the Terminating Hospital as a primary hospital, but have admitted enrollees for the most recently completed calendar year, are able to refer their enrollees to other hospitals within the Plan's network for services comparable to those provided by the Terminating Hospital.

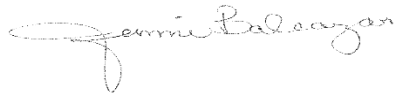
12. Please identify the number of enrollees who have a procedure scheduled and authorized by the Plan at the Terminating Hospital post-termination.

Plan Response12: To date, 23 enrollees have an authorized scheduled procedure within 180 days of the contract termination date.

Please submit the Plan's response by **March 10, 2026**, via the Block Transfer portal as a "Response to Comment Letter."

Please contact me if there are any questions regarding the above.

Sincerely,

A handwritten signature in cursive script, reading "Jennie Balcazar". The signature is written in dark ink on a white background.

Jennie Balcazar
Analyst I
Office of Plan Monitoring